

CVD RISK ASSESSMENT CHART

PARAMETERS REQUIRED PRIOR TO USING THE CHART:

Age, Sex, Current smoking status, Presence or absence of diabetes, Systolic blood pressure, Total cholesterol

STEP 1 Select the appropriate chart depending on the availability of blood cholesterol report.

STEP 2 Select the appropriate chart depending on the presence or absence of diabetes.

STEP 3
Select male or female tables.

STEP 4
Select smoker or non smoker.

STEP 5 Select age group box (if age is 40-49 years select 40, if age is 50-59 years select 50, if age is 60-69 years select 60, if age is 70-79 years select 70).

STEP 6 Within this box find the nearest cell where the individuals systolic blood pressure (mm Hg) and total blood cholesterol level (mmol/l).

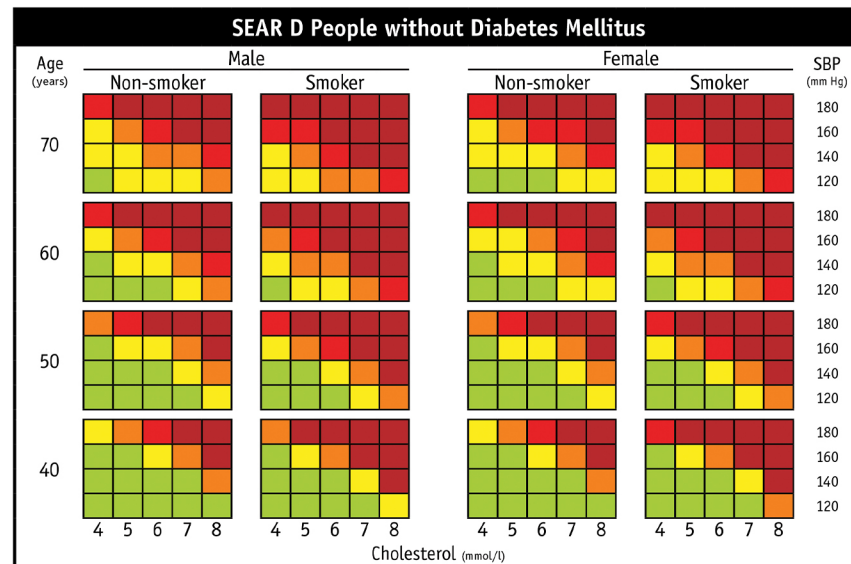
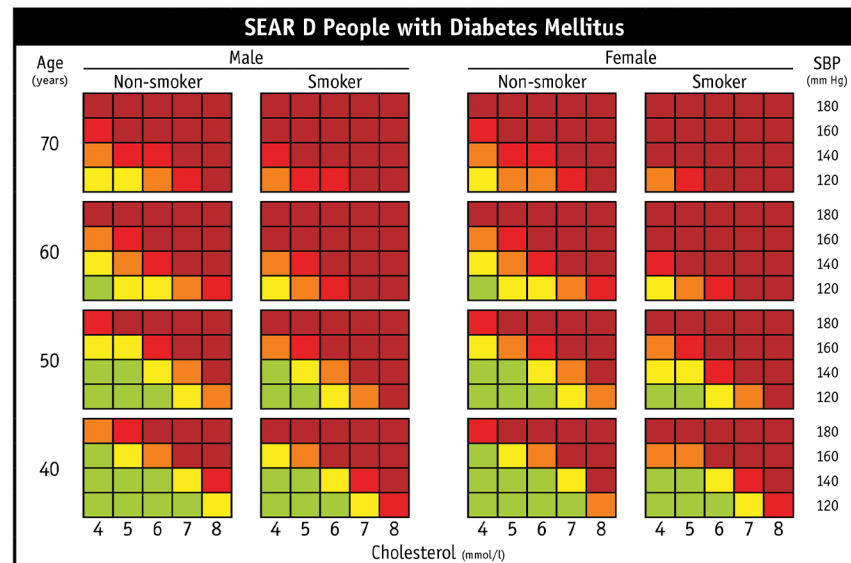
THE COLOUR OF THIS CELL DETERMINES THE 10-YEAR CARDIOVASCULAR RISK.

* Fasting plasma glucose ≥ 126 mg/dl, or 2-h plasma glucose ≥ 200 mg/dl, or HbA1c $\geq 6.5\%$, or known diabetes

** Cholesterol values are to be entered in the chart as mmol/L.

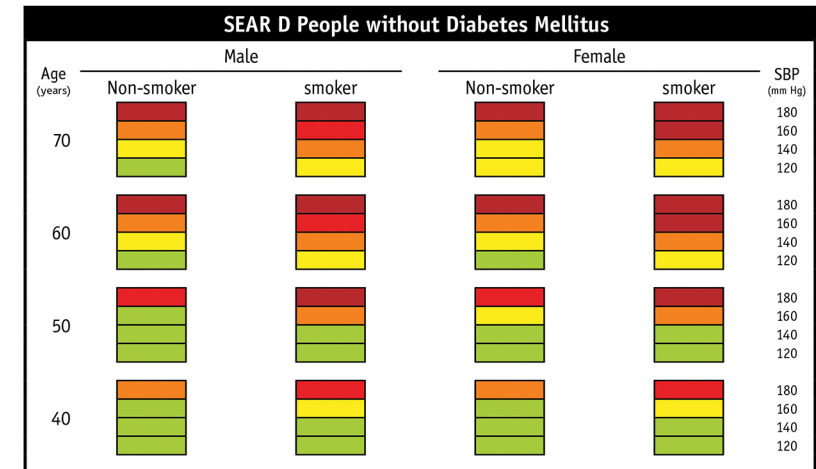
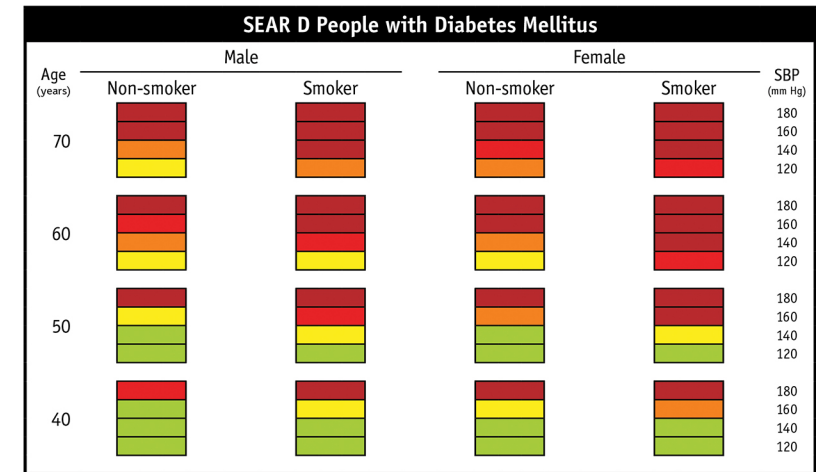
To convert cholesterol mg/dl to mmol/L, multiply by 0.02586 Example: TC: 200 mg/dl $\times 0.02586 = 5.172$ mmol/L

Risk Level ■ <10% ■ 10% to <20% ■ 20% to <30% ■ 30% to <40% ■ $\geq 40\%$



USE THE FOLLOWING CHART IF BLOOD CHOLESTEROL REPORT NOT AVAILABLE

Risk Level ■ <10% ■ 10% to <20% ■ 20% to <30% ■ 30% to <40% ■ $\geq 40\%$



TERMINOLOGIES:

TIA: Transient Ischemic Attack.

ACEi: ACE inhibitor.

ARB: Angiotensin Receptor Blocker.

GFR: estimated glomerular filtration rate.

Disclaimer: This leaflet is currently being piloted and may contain inconsistencies. Please help us improve it by reporting any issues to KHDC Nepal.



**KHDC
NEPAL**



THE FOLLOWING POPULATION HAVE CLINICALLY DETERMINED HIGH RISK OF CARDIOVASCULAR DISEASE, AND **DO NOT** NEED RISK ASSESSMENT:

- Known cardiovascular disease (Ischemic heart disease, Transient Ischemic Attack, Stroke, Peripheral Artery Disease)
- Moderate-to-severe chronic kidney disease (eGFR $<45\text{mL/min/1.73m}^2$)*
- Diabetes with age equal to or more than 40 years or diabetes with protein in urine*
- Previous diagnosis of familial hypercholesterolemia
- Persistently elevated systolic BP >160 mm Hg; or diastolic BP >100 -105 mm Hg, or those with albuminuria, left ventricular hypertrophy in ECG or hypertensive retinopathy
- Total cholesterol* >320 mg/dl or LDL >190 mg/dl
*(if available)

CARDIOVASCULAR DISEASE [CVD] RISK ASSESSMENT

FOR PRIMARY CARE PROVIDERS

2026/05/06 V 5.0

OUR COLLABORATORS



HUG Hôpitaux
Universitaires
Genève

Ref. PEN Manual (MoHP), 2075

CONTACT US

For any clinical queries / If anything in the document is difficult to understand



+977-9852072088 / 9802766889

www.khdcnepal.org



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IDENTIFY PEOPLE FOR CVD RISK ASSESSMENT

Age 40-79 years old and **NOT** fulfilling high risk criteria



ASSESS CVD RISK

Use WHO/ ISH risk chart SEAR-D version for Nepal



IDENTIFY THE RISK CATEGORY

RISK LEVEL:

$>20\%$

10-20%

5-10%

$<5\%$

COUNSEL

- Counsel on diet (which includes lipid-lowering diet), physical activity, smoking cessation and avoiding harmful use of alcohol.

TREAT

REFER TO KHDC HYPERTENSION LEAFLET

LIPID-LOWERING DRUGS (STATINS)

- Give atorvastatin 10-20 mg.

FOLLOW-UP

- Follow up every 3 months.
- If there is no reduction in Cardiovascular risk after 6 months of follow-up, refer to next level.
- Follow up every 3-6 months.
- Follow up every 3 months until targets are met, then 6-9 months thereafter.
- Follow up in 12 months if treatment is not initiated.



MANAGE CVD RISK

- Manage hypertension, diabetes mellitus, CKD and advice on avoiding chewing tobacco, smoking cessation according to individual KHDC leaflets
- Prescribe Atorvastatin 10-20 mg once daily if CVD risk more than 20%
- Treat albumin in urine* with ACEi (Enalapril 2.5 mg BD) or ARB (Losartan 25 mg OD) and refer for evaluation
- Advice on physical activity, abstinence of alcohol, diet and stress management
*(if available)